

University of Maryland - CS3

Maryland Next Gen NCLEX Test Bank Project September 1, 2022; Author: Kadriyya Clark, DNP, RN, CNE, Community College of Baltimore County. Acute respiratory failure (medical-surgical): a 78-year-old client in the ICU with pneumonia progresses to intubation and mechanical ventilation.

Taken from the Maryland Next Gen NCLEX Test Bank Project, University of Maryland School of Nursing: "Acute Respiratory Distress" medical-surgical faculty case study (Kadriyya Clark, DNP, RN, CNE, Community College of Baltimore County, September 1, 2022). Available at <https://www.nursing.umaryland.edu/mnwc/initiatives/nextgen-nclex/nextgen-nclex-library/> (Faculty Case Studies). Laboratory values shown in SI units.

Draft review. Each screen scores full marks with its answer key and keeps every chart tab and entry on later screens. Added hidden from students (draft) until reviewed. Each screen is shown as students see it, then answered with the key; the rationale follows.

Chart tabs (last screen)

NOT COMPLETE

Case Study Screen 6 of 6

The nurse cares for a 78-year-old female client admitted to the medical intensive care unit in respiratory distress.

Nurses' Notes

Laboratory Results

Orders

Laboratory Test and Reference Range	0900
Arterial pH 7.35–7.45	7.20
PaO₂ 75–100 mm Hg	75 mm Hg
PaCO₂ 35–45 mm Hg	51 mm Hg
SaO₂ 95–100%	96%
Bicarbonate (HCO₃⁻) 22–26 mmol/L	28 mmol/L
White blood cell (WBC) count 4.5–10.5 × 10 ⁹ /L	35.0 × 10 ⁹ /L
Platelet count 140–450 × 10 ⁹ /L	250 × 10 ⁹ /L
Potassium 3.5–5.0 mmol/L	4.0 mmol/L

NOT COMPLETE

Case Study Screen 6 of 6

The nurse cares for a 78-year-old female client admitted to the medical intensive care unit in respiratory distress.

Nurses' Notes

Laboratory Results

Orders

Category	Orders
Nursing	Insert urinary catheter Suction endotracheal tube as needed Titrate oxygen to keep pulse oximetry reading at or above 95%
Medications	Start IV of 0.9% sodium chloride at 75 mL/hr Amoxicillin 500 mg IVPB every 12 hours Midazolam 2–4 mg IV push every 1 hour as needed for agitation Acetaminophen 650 mg per rectum every 8 hours as needed for T > 100.8°F (38.2°C)
Monitoring	Perform 12-lead ECG Call for chest X-ray Blood gas in 30 minutes

Screen 1 of 6 · Recognize cues · select_n

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse cares for a 78-year-old female client admitted to the medical intensive care unit in respiratory distress.

Nurses' Notes

Laboratory Results

0900: Client was admitted to the ICU in respiratory distress after minimal response to high-flow oxygen for a pulse oximetry reading of 83% on room air. Crackles heard bilaterally in lower lobes with diminished breath sounds in right middle lobe; S1 S2 auscultated; bowel sounds positive in all 4 quadrants; skin warm, dry, and intact. VS: T 99.8°F (37.7°C); HR 110; RR 30; BP 168/90; pulse oximetry reading 87% on 100% non-rebreather mask; pain 0/10.

Select the 4 findings that are most urgent.

☐ 1. BP 168/90

☐ 2. WBC 35.0 × 10⁹/L

☐ 3. Lactate 4.5 mmol/L

☐ 4. pH 7.20

☐ 5. PaCO₂ 51 mm Hg

☐ 6. PaO₂ 75 mm Hg

☐ 7. Respiratory rate 30

☐ 8. Blood culture: gram-negative cocci

☐ 9. Pulse oximetry reading 87%

Submit

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Case Study Screen 1 of 6

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Rationale: The client has signs and symptoms of infection and respiratory failure. The most urgent findings are related to respiratory failure: a pH of 7.20 indicating an acidic state, a PaCO₂ above 50 mm Hg, tachypnea (RR 30), and a pulse oximetry reading of 87% on high-flow oxygen. The blood pressure is elevated but is not yet critical.

Screen 2 of 6 · Analyze cues · matrix_mr

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

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Case Study Screen 2 of 6

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➤ For each finding, click to specify if the finding is consistent with acute respiratory failure or pneumonia. Each finding may support more than one condition.

Finding	Acute Respiratory Failure	Pneumonia
Crackles in bilateral lower lobes	☐	☐
Pulse oximetry reading 87% on 100% oxygen	☐	☐
PaCO ₂ 51 mm Hg	☐	☐
pH 7.20	☐	☐

Note: Each column must have at least 1 response option selected.

Submit

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Calculator

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Case Study Screen 2 of 6

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Crackles in bilateral lower lobes	☒ ✓	☒ ✓
Pulse oximetry reading 87% on 100% oxygen	☒ ✓	☒ ✓
PaCO ₂ 51 mm Hg	☒ ✓	☐
pH 7.20	☒ ✓	☐

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Rationale: Bilateral crackles can be associated with both conditions and are common in respiratory illnesses. A pulse oximetry reading of 87% indicates a low level of oxygen in the blood, which can be caused by either condition. A PaCO₂ of 51 mm Hg and a pH of 7.20 indicate respiratory acidosis and are consistent with acute respiratory failure.

Screen 3 of 6 · Prioritize hypotheses · multiple_choice

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

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What does the nurse determine is the priority for this client's care?

☐ 1. Administering corticosteroids to reduce inflammation

☐ 2. Intubating for mechanical ventilation support

☐ 3. Opening airways with aerosol treatments

☐ 4. Treating infection with antibiotics

Submit

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Question Navigator

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Case Study Screen 3 of 6

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Rationale: Acute respiratory failure is characterized by the lungs' inability to oxygenate properly. The client has had a poor response to maximum supplemental oxygen and now needs mechanical ventilation. The respiratory failure was most likely caused by pneumonia; treating the pneumonia should take place next. Aerosols and corticosteroids may be incorporated into the treatment plan but are not as critical as intubation.

Screen 4 of 6 · Generate solutions · matrix_mc

Practice NCLEX RN Test

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Calculator

Question Navigator

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Case Study Screen 4 of 6

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0930:

Client was intubated with a #6 endotracheal tube and placed on assist-control mechanical ventilation, rate 14, PEEP 5 cm H₂O, FIO₂ 60%. VS: T 99.8°F (37.7°C); HR 110; RR 14; BP 180/90; pulse oximetry reading 96% on FIO₂ 60%. Client is restless.

The nurse has reviewed the Nurses' Notes from 0930. The nurse plans care for the client after beginning mechanical ventilation.

➤ For each potential intervention, click to specify whether the intervention is indicated or not indicated to include in the plan of care.

Potential Intervention	Indicated	Not Indicated
Administer sedatives	☐	☐
Repeat chest X-ray	☐	☐
Schedule suctioning every 2 hours	☐	☐
Administer amiodarone	☐	☐
Administer IV antibiotics	☐	☐
Position supine with head midline	☐	☐
Obtain an electrocardiogram (ECG)	☐	☐

Note: Each row must have only 1 response option selected.

Submit

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Case Study Screen 4 of 6

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Position supine with head midline	☐	☒
Obtain an electrocardiogram (ECG)	☒	☐

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Rationale: Most intubated clients require sedation to prevent them from fighting the ventilator. A chest X-ray should be done after intubation to confirm endotracheal tube placement. Antibiotics are indicated to fight infection. The client has tachycardia and an elevated blood pressure, which makes obtaining an ECG important. Suctioning can damage tracheal tissue and should be done as needed, not on a schedule. Amiodarone, an antiarrhythmic, is not indicated because no arrhythmia has been identified. Positioning the client supine is not indicated; semi-Fowler's or prone positioning is best to help with postural drainage.

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Practice NCLEX RN Test

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Calculator

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NOT COMPLETE

Case Study Screen 5 of 6

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The nurse has reviewed the Laboratory Results and receives orders.

Click to highlight the 3 orders the nurse should implement first.

Select 3 correct findings.

Orders

Category	Orders
Nursing	Insert urinary catheter
	Suction endotracheal tube as needed
	Titrate oxygen to keep pulse oximetry reading at or above 95%
Medications	Start IV of 0.9% sodium chloride at 75 mL/hr
	Amoxicillin 500 mg IVPB every 12 hours
	Midazolam 2–4 mg IV push every 1 hour as needed for agitation
Monitoring	Acetaminophen 650 mg per rectum every 8 hours as needed for T > 100.8°F (38.2°C)
	Perform 12-lead ECG
	Call for chest X-ray
	Blood gas in 30 minutes

Submit

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Case Study Screen 5 of 6

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Nursing	Insert urinary catheter
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Medications	Start IV of 0.9% sodium chloride at 75 mL/hr ✓
	Amoxicillin 500 mg IVPB every 12 hours
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Monitoring	Acetaminophen 650 mg per rectum every 8 hours as needed for T > 100.8°F (38.2°C)
	Perform 12-lead ECG
	Call for chest X-ray ✓
	Blood gas in 30 minutes

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Rationale: A chest X-ray is needed after intubation to confirm optimal endotracheal tube placement. The client does not yet have venous access; this should be established immediately so medications can be given. The BP and heart rate are significantly elevated, and the client is agitated; sedation (midazolam) should be given to decrease agitation and the risk of self-extubation.

Next, an ECG can be done and antibiotics can be given. The oxygen level is above 95%, so adjustments are not needed, and there is no indication that suctioning is needed. The blood gas is not due yet. The urinary catheter can be placed after other treatments.

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Practice NCLEX RN Test

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Case Study Screen 6 of 6

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1000: Chest X-ray obtained. IV placed and midazolam given. ECG shows sinus tachycardia. VS: T 99.8°F (37.7°C); HR 100; RR 14; BP 150/70; pulse oximetry reading 93% on FIO₂ 60%; drowsy, -1 on sedation scale.

The nurse has reviewed the Nurses' Notes from 1000 and the Orders. The nurse reassesses the client at 1000 and compares the findings to 0930.

➤ For each finding, click to specify if the finding indicates that the client's status has improved, declined, or is unchanged.

Finding	Improved	Declined	Unchanged
Heart rate	☐	☐	☐
Temperature	☐	☐	☐
Respiratory rate	☐	☐	☐
Blood pressure	☐	☐	☐
Pulse oximetry reading	☐	☐	☐
Agitation	☐	☐	☐

Note: Each row must have only 1 response option selected.

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Finish attempt →

Practice NCLEX RN Test

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Calculator

Question Navigator

COMPLETE

Case Study Screen 6 of 6

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Agitation	☒	☐	☐

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Rationale: The heart rate decreased from 110 to 100 and the blood pressure dropped from 180/90 to 150/70; both indicate improvement. The client is drowsy, showing that the agitation has decreased. The temperature and respiratory rate remain unchanged. The pulse oximetry reading has declined slightly (96% to 93%); the nurse should assess the lung sounds to determine if suctioning or ventilator changes are needed.